

3.11 Discontinuation of the Study

The study may be stopped if, in the judgment of AstraZeneca, trial patients are placed at undue risk because of clinically significant findings that:

- meet individual stopping criteria or are otherwise considered significant
- are assessed as causally related to study drug
- are not considered to be consistent with continuation of the study

Regardless of the reason for termination, all data available for the patient at the time of discontinuation of follow up must be recorded in the CRF. All reasons for discontinuation of treatment must be documented.

In terminating the study, the Sponsor will ensure that adequate consideration is given to the protection of the patients' interests.

4. STUDY PLAN AND TIMING OF PROCEDURES

The schedule of assessments for the screening visit is shown in [Table 1](#). On-study assessments are shown in [Table 2](#).

Table 1 Study Schedule – Screening (Visit 1)

Day	Before screening period	-28 to -1
Informed consent	X	
IVRS/IWRS unique patient number obtained	X	
Demographics		X
Medical and surgical history		X
Prior cancer therapies including radiotherapy, best response to prior chemotherapy regimen		X
History of blood transfusions ^a		X
Inclusion/exclusion criteria		X
ECOG performance status		X
Physical examination		X
Vital signs (includes blood pressure, pulse and temperature), body weight		X
Haematology / clinical chemistry ^{b,f}		X
Urinalysis		X
Pregnancy test for women of childbearing potential ^c		X
Confirmed as having non-Germline <i>BRCA</i> Mutated ovarian cancer ^d	X	

Day	Before screening period	-28 to -1
Tumour assessment ^e		X
AEs (from time of consent)		X
Concomitant medications		X
Archival or fresh tumour biopsy sample for post treatment analysis confirmation of HRD status ^g	X	X

^a Include history of blood transfusion within previous 120 days from start of study treatment and the reasons, e.g., bleeding or myelosuppression.

^b Coagulation test should be performed if clinically indicated. For a list of all required laboratory tests please refer to Section 5.2.1.

^c Women of childbearing potential must have a negative urine or serum pregnancy test within 28 days prior to starting treatment and a confirmatory test before treatment on Day 1. If results are positive, the patient is ineligible/must be discontinued from the study.

^d Patients must be non-germline *BRCA* mutated to be enrolled into the study. Patients for whom their *gBRCA* status is already known should be consented to the study within 28 days prior to Day 1 of study treatment. Patients who do not know their mutation status will need *BRCA1/2* mutation screening (see Section 4.1.1 and Appendix F).

^e Includes CA-125 measurement where required (Refer to Inclusion Criterion 6). RECIST assessment should be performed no more than 28 days before the start of study treatment, and ideally should be performed as close as possible to the start of study treatment.

Note: MRI/ CT scan more than 28 days prior to Day 1 may be acceptable, please consult with AstraZeneca.

^f These measurements may be repeated within the screening period if abnormalities are found which are the only reason for screen failing the patient and which can reasonably be expected to have return to the acceptable range

^g All patients are required to submit either an archival tumour sample, or a fresh biopsy if an archival sample is not available, for retrospective testing of their genetic status. If archival tissue, the presence of adequate tissue needs to be confirmed prior to screening, but it will not be shipped to the central lab before Day1. If fresh, the biopsy should be taken after the baseline scan has been performed. Samples should be submitted as paraffin blocks, alternatively, sections mounted on glass slides prepared from the block can be provided. Please refer to the laboratory manual for further details regarding tissue collection, shipping and storage.

Table 2 Study Plan Detailing the Procedures

Visit Number or type	2	3	Visit No. 4 and subsequent tumour assessment visits	Visit No. 5 and subsequent safety visits ^a (For the first 12 months only)	Study treatment discontinued	Follow up 30 days after last dose of study medication	Long-term follow up
Day	1	29	On the first day of next visit period (V4 = Day 57 and thereafter every 8 weeks for the first 12 months; every 12 weeks thereafter ^a)	On the first day of next visit period (V5=Day 85) and thereafter every 8 weeks)			
Visit Window	0	±3d	±7d	±3d	±7d	+7d	
Physical examination ^c	X ^b						
Vital signs, body weight (includes blood pressure, pulse and temperature) ^c	X ^b						
ECOG performance status	X		X		X		
Haematology / clinical chemistry ^d	X ^b	X	X	X	X	X	
Urinalysis ^c	X ^b						
Pregnancy test ^e	X	X	X	X		X	
Tumour assessment ^f			X				
Blood sample for ctDNA	X				X		
Blood sample for retrospective gBRCA test ^g	X						
AEs ^h	X	X	X	X	X	X	
FACT-O and EQ-5D-5L questionnaires	X	X	X		X	X	

Visit Number or type	2	3	Visit No. 4 and subsequent tumour assessment visits	Visit No. 5 and subsequent safety visits ^a (For the first 12 months only)	Study treatment discontinued	Follow up 30 days after last dose of study medication	Long-term follow up
Day	1	29	On the first day of next visit period (V4 = Day 57 and thereafter every 8 weeks for the first 12 months; every 12 weeks thereafter ^a)	On the first day of next visit period (V5=Day 85) and thereafter every 8 weeks)			
Visit Window	0	±3d	±7d	±3d	±7d	+7d	
Concomitant medications including blood transfusions	X	X	X	X	X	X	
Olaparib dispensed/returned	X	X ^j	X ^j		X		
Subsequent cancer therapy following discontinuation of study treatment ^k						X	X
Time to subsequent therapy and Survival ^l							X

^a From Visit 3 onwards, visits will take place on the first day of each 4-week visit period, relative to the date of the first olaparib dose (if the patient has not progressed and is still on treatment). Where the visit does not coincide with tumour assessment visits, these safety visits will only be for laboratory and pregnancy testing and recording of ECOG status, AEs and concomitant medication including transfusions. Where these visits coincide with tumour assessments, the assessment visit schedule will be performed

^b If assessed within 7 days before first dose and meets the stated eligibility criteria (if applicable), it does not need to be repeated on Day 1 of study treatment unless Investigator believes that it is likely to have changed significantly.

^c To be additionally performed if clinically indicated at any other time.

^d All samples should be taken prior to first dose. Coagulation test should be performed if clinically indicated. For a list of all required laboratory tests please refer to Section 5.2.1.

^e Pregnancy tests on blood or urine samples will be performed for women of childbearing potential within 28 days prior to the start of study treatment, on Day 1 of the study prior to commencing treatment, at the time points shown in Table 2 during study treatment and at the 30-day follow-up visit. If results are positive the patient is ineligible/must be discontinued from study treatment immediately.

^f Visit 4 onwards: Subsequent tumour assessments will be conducted every 8 weeks (±7 days) for the first 12 months and then every 12 weeks (± 7 days) until documented disease progression.

- ^g Unless patient was tested at Myriad as part of screening. All patients are required to provide a 9 ml blood sample for confirmation of *gBRCA* status. Please refer to the laboratory manual for further details regarding blood sample collection, shipping and storage.
- ^h All ongoing AEs/SAEs and any new AEs/SAEs identified during the 30 calendar days follow-up period after last dose of study medication must be followed to resolution.
- ^j +/- 2 days visit window allowed for the dispensing. Sufficient study treatment should be dispensed for at least each treatment period plus overage; however, additional treatment can be dispensed to patients to last longer if required.
- ^k Start date of the first anticancer treatments (including, but not limited to, chemotherapy and targeted agents), following discontinuation from study treatment must be recorded.
- ^l Patient will be followed (can be by phone or email) regularly until start of subsequent therapy; survival will be followed at approximately 12 weekly intervals thereafter.